COMMONWEALTH OF MASSACHUSETTS

CONOR L. PROFFITT,	)	
COMPLAINANT	)	
	)	**MCAD NO. 21-BEM-01753**
V.	)	
	)	**EEOC NO. 16C-2021-01559**
BOSTON CHILDREN'S HOSPITAL,	)	
RESPONDENT	)	

POSITION STATEMENT OF RESPONDENT
BOSTON CHILDREN'S HOSPITAL

I. INTRODUCTION

Boston Children's Hospital (the "Hospital", "BCH" or "Respondent") is a charitable corporation primarily engaged as an academic medical center focused on medical research and the medical care of children. BCH is a full service medical center open 24 hours a day, every day. The Children's Hospital Applications Maximizing Patient Safety program (the "CHAMPS Program") is responsible for promoting efficiency and safe patient care by implementing clinical applications and completing projects that allow third party applications to interface with the Hospital's electronic medical record system. The CHAMPS program is where Mr. Proffitt's ("Mr. Proffitt" or "Complainant") allegations arise. *See* Charge of Discrimination, attached hereto as **Exhibit A** ("Charge"). Mr. Proffitt's claims of race and/or color discrimination, as well as retaliation, are without merit. As set forth in more detail below:

- Mr. Proffitt – a 33 year-old white male – experienced significant and documented performance challenges beginning early into his employment with the Hospital. The Hospital attempted to counsel Mr. Proffitt on his performance through its progressive discipline system. However, Mr. Proffitt failed to improve. Ultimately, the Hospital was forced to separate Mr. Proffitt's employment after his performance issues led to repeated customer complaints and caused significant delays in his work, postponing critical projects that could have improved the delivery of care to the Hospital's vulnerable pediatric population.
- Mr. Proffitt's purported complaint to HR was made *on the very day that* Mr. Proffitt received a Final Written Warning (the third formal step in his disciplinary process) for his performance issues. Moreover, the purported conversation referenced by Mr. Proffitt in his compliant allegedly occurred approximately six months prior. Mr. Proffitt's complaint was fully reviewed and found to be completely without merit. Mr. Proffitt's report was not made in good faith, and was a transparent attempt to frustrate the progressive corrective action process, harm the manager managing his harassment

and performance issues, and potentially create legal cover for himself after it became clear that his employment was in jeopardy.

- Mr. Proffitt's allegations of race discrimination are particularly misplaced in the context of Boston Children's Hospital full-fledged commitment to the principles of equity, diversity and inclusion. Moreover, he has offered no comparator, no evidence, nor any reason how he, as a white man, was discriminated against on the basis of his race and/or color.

The Charge is legally and factually deficient, and it should be dismissed.

II. BACKGROUND

A. The CHAMPS Program at Boston Children's Hospital.

The Children's Hospital Applications Maximizing Patient Safety program is a subdivision of the Information Services Department. The CHAMPS program has multiple project teams, performing different functions. One of those project teams is responsible for optimizing and updating the connection between the Hospital's electronic medical record system and third party applications that interface with the medical record. Put simply, when the Hospital determines it needs to use a new application, whether to promote patient safety or for some other reason, it is the CHAMPS program that is responsible for ensuring that this new application works appropriately with the Hospital's electronic medical record system.

Applications Development Specialists IIIs are responsible for taking these application projects, and driving them to completion by researching, designing, and implementing the technical builds that allow the application to interface with the Hospital electronic medical record system. *See* Applications Development Specialists III Job Description, attached hereto as **Exhibit B**. The Application Development Specialist III role is critical because the projects they are assigned, when completed, will often result in increased Hospital efficiency and patient safety. When these projects languish, or go uncompleted, the Hospital's clinicians are not being provided the best available tools to serve the Hospital's pediatric patients.

B. Mr. Proffitt's First Year Challenges at Boston Children's Hospital.

On May 20, 2019, Mr. Proffitt was hired into the CHAMPS program as an Electronic Medical Record Analyst. **Charge** ¶ 2; *see* New Hire Letter, dated as of April 9, 2019, attached hereto as **Exhibit C**. Early into Mr. Proffitt's employment, Mr. Proffitt's Analyst position was re-labeled as an "Applications Development Specialist III" as part of a Hospital-wide title consolidation program; however, Mr. Proffitt's job responsibilities remained fundamentally the same. As an Application Development Specialist, Mr. Proffitt reported to Travis Rossner (IT Manager I), who in turn, reported to Bradley McDonald (Director, Core Clinical Applications).

Mr. Rossner knew Mr. Proffitt before Mr. Proffitt was hired by the Hospital. In fact, Mr. Rossner suggested hiring Mr. Proffitt because they had previously worked together at Cerner, the proprietor of the electronic medical record system used by Boston Children's Hospital. Mr. Proffitt appeared to be an ideal fit for Mr. Rossner's team given Mr. Proffitt's apparent familiarity with the Cerner medical record system.

Nevertheless, shortly into Mr. Proffitt's employment with BCH, he began engaging in unprofessional behavior and was struggling to adequately to perform the functions of his position. In December 2019, a female co-worker alleged that Mr. Proffitt was engaging in a pattern of harassing her. *See* Mr. Rossner's Internal Notes on Performance Concerns, attached hereto as **Exhibit D**. Mr. Rossner, Mr. Proffitt's manager, verbally counseled Mr. Proffitt on unprofessional interactions in the workplace due to the complaint. In January 2020, as part of Mr. Proffitt's mid-year review, Mr. Rossner provided Mr. Proffitt written counseling, which indicated that Mr. Proffitt was struggling in three basic areas: (1) he appeared uninterested and unaccountable for completing certain projects that were assigned to him, often failing to take key next steps; (2) he was uncommunicative about his projects and their statuses, which often led CHAMPS management to be unclear on where Mr. Proffitt's projects stood; and (3) there were repeated reports of interpersonal challenges with Mr. Proffitt's teammates, as Mr. Proffitt would become condescending, dismissive, and hostile when there was a slight difference of opinion regarding how to approach a project. *See* Proffitt ISD Core Competency Template - Staff Level III, attached hereto as **Exhibit E**.

After these issues were addressed in Mr. Proffitt's January 2020 mid-year review, his performance concerns continued. *See* **Exhibit D**. In February 2020, Mr. Proffitt was counseled by Mr. Rossner for his continued lack of engagement on critical projects, as well as failing to show up to key team meetings. *Id.*; *see also* February 2020 Email Regarding Missed Huddle, attached hereto as **Exhibit F**. In March 2020, Mr. Rossner counseled Mr. Proffitt again after he failed to take requested steps in a key project, and was reported to have engaged in an unprofessional interpersonal interaction; in this counseling meeting, Mr. Proffitt refused to take ownership of assigned work that was incomplete. *See* **Exhibit D**.; Aggregated March 2020 Email Regarding Incomplete Tasks, attached hereto as **Exhibit G**.

In the spring of 2020, Mr. Proffitt received a round of reviews from key individuals in his area: Dr. Jonathan Bickel (the Information Technology Physician Lead for the Hospital); Bradley McDonald (Director of Core Clinical Applications and Mr. Rossner's supervisor); and Brenda Dodson (Electronic Health Record Architect). The reviews were solicited as part of the CHAMPS program's standard practice for employees who have an upcoming annual review. The feedback from the reviewers was consistent, clear, and unequivocal. Mr. Proffitt had engaged in a consistent pattern of unprofessional interpersonal interactions; a demonstrable apathy towards resolving key projects, with inadequate communication that led critical initiatives floundering; and a lack of accountability over his projects' statuses. *See* Conor Proffitt Aggregated 360 Reviews, attached hereto as **Exhibit H**.

In May 2020, Mr. Rossner was due to provide Mr. Proffitt his annual performance review to reflect Mr. Proffitt's first year of work for the Hospital. On this review, Mr. Proffitt received the lowest possible score of a "1" (on a scale of 1-5), indicating that Mr. Proffitt "Need[ed] Immediate Improvement." *See* Annual Performance Review, attached hereto as **Exhibit I**. Mr. Proffitt's managers summarized his performance as follows:

> *Overall this has been a tumultuous year for Conor in regards to integrating into the BCH culture, his willingness to communicate, tickets being logged properly and completely and his ability to provide documentation once the project build has been completed, all of which has not met BCH standards. It seems as if Conor does not have the ability to work well with others, . . . [and] it seems that anyone who asks Conor to do something or has to hold him accountable for work he does not have strong relationships with . . . . I truthfully*

hope we can improve on all the categories [we have] addressed and can leave this behind us. Each of the issues we addressed in January during your core competency, have not changed including your demeanor, communication, or work product. This must change immediately and we as leaders will be here to help and guide you so if you have any questions on what needs to be done or expectations we will make those clear.

Id. Under standard Boston Children's Hospital practice, when an employee is rated an overall "1" on their annual performance review, the employee is simultaneously provided a Performance Improvement Notice/Final Written Warning, which highlights the areas of needed improvement, and steps necessary to improve. *See* Performance Evaluation Policy, attached hereto as **Exhibit J**. Accordingly, in connection with Mr. Proffitt's Performance Review, he was also provided a Performance Improvement Notice/Final Written Warning on July 9, 2020. *See* Performance Improvement Notice/Final Written Warning, attached hereto as **Exhibit K**.

The Final Written Warning reflected the same concerns that had previously been identified with Mr. Proffitt, namely, his failure to appropriately resolve and communicate on projects leading to customer complaints; and challenges regarding an unprofessional and confrontational demeanor. *Id.* The PIN/Final Warning enumerated seven specific areas that needed improvement:

1. *Maintain a professional demeanor and eliminate confrontational interactions.*
2. *Timely response and acknowledgement to requests or communications from leaders or peers is expected without unreasonable questioning and resistance.*
3. *Improved performance will be measured in part by decreased customer complaints regarding errors and misjudgments*
4. *Productivity, involving work of equivalent complexity should be consistent without decline.*
5. *Demonstrate pro-active communication to peers and leaders regarding work assignments.*
6. *Demonstrate the ability to effectively self-manage daily work assignments, Completing Wiki documentation fully in a timely manner / before project go-lives, and meeting establish deliverable dates based on workload, and communicate pro-actively to management owner.*
7. *Improved and advanced communication is expected if you are meeting with obstacles preventing you from meeting a project deadline.*

Id. Mr. Proffitt's performance challenges had led to delays in critical projects, denying Hospital clinicians the best available tools for delivering pediatric care. For that reason, among many others, Mr. Proffitt's supervisory team committed to trying to assist Mr. Proffitt in improving his performance. The PIN/Final Written Warning also indicated that they would endeavor to support Mr. Proffitt. *Id.* (including by providing regular meeting and feedback). By all accounts, Mr. Proffitt's leadership team wanted Mr. Proffitt to succeed.

C. Mr. Proffitt's Grievance

On the very same day that Mr. Proffitt received his Final Written Warning, Mr. Proffitt called Human Resources to file a grievance (the "Grievance") pursuant to Boston Children's Hospital's "Grievance" Policy. *See* Proffitt Grievance Review Request, attached hereto as **Exhibit L**; Boston Children's Hospital Grievance Policy, attached hereto as **Exhibit M**. Mr. Proffitt's Grievance largely tracks the substance of the Charge here. In the Grievance, Mr. Proffitt alleged that in a conversation, which occurred approximately six months prior, Mr. Rossner stated to him "I don't see why we have to take Martin Luther King Day off" and "I wish we didn't have to take Martin Luther King Day off." **Exhibit L**; **Charge** ¶ 3. In Mr. Proffitt's Grievance, he stated that

he did not respond to these alleged comments in the moment, and that the first time he raised any concerns about the comments to either a supervisor or Human Resources was over six months later, only after he had received his Final Written Warning. *See* **Exhibit L**. In his Grievance form, Mr. Proffitt explained that "I fully expect the company to protect me from retaliation based on the filing of this grievance . . . ." *Id.*

Upon receipt of the Grievance, and according to the Hospital's Grievance Review Policy, the Hospital's Director of Employee Relations assigned an Employee Relations advisor to act as a Grievance Coordinator and review Mr. Proffitt's claim. The Grievance Coordinator reviewed Mr. Proffitt's Grievance form; conducted an interview with him; and also interviewed additional parties relevant to the Grievance, including Mr. Rossner and Mr. McDonald. When the Grievance Coordinator spoke to Mr. Proffitt, he heavily emphasized a desire to change supervisors.

Ultimately, the Grievance Coordinator found that Mr. Proffitt's Grievance was not supported. The Grievance Coordinator credited Mr. Rossner's account of the events, finding that Mr. Rossner's comments were not an expression of racism or animus towards Martin Luther King, Jr. Rather, Mr. Rossner's department was busy and he disagreed with the Hospital's policy that required employees to take a vacation day on Hospital holidays (without seeking managerial approval). Ultimately, Human Resources informed Mr. Proffitt that he was not eligible to transfer to a new supervisory chain due to a Hospital policy that restricts transfers when an employee is subject to a Final Written Warning. *See* BCH Transfer Policy, attached hereto as **Exhibit N**. Mr. Proffitt chose not to appeal the decision, which he was permitted to do under the Hospital's Grievance Review Policy. *See* **Exhibit M**.

D. Mr. Proffitt Fails to Improve His Performance.

After Mr. Proffitt was issued his Final Written Warning on July 9, 2020, his performance did not improve. In fact, from July 9, 2020 through November 2020, Mr. Proffitt failed to make even marginal improvement in the areas of concern that had been identified by his leadership.

- *Resolution of Key Assignments* – Mr. Proffitt continued to be apathetic and uncommunicative around resolving key assignments. *See, e.g.*, Email Correspondence, dated as of August 11, 2020 and October 7, 2020, attached hereto as **Exhibit O**. Mr. Proffitt's failure to complete his work required reallocation of departmental resources to help resolve his many outstanding projects, and simple work assignments often required multiple revisions due to lack of attention to basic detail (such as work being performed for the wrong department). *See, e.g.*, Brad McDonald Update on September 18, 2020, and Ticket Delay, dated as of September 20, 2020, attached hereto as **Exhibit P**.
- *Interpersonal Interactions* – Mr. Proffitt's supervisors continued to receive complaints from employees, customers, and vendors regarding his demeanor. Mr. Proffitt was also dismissive of team meetings, routinely missing them and offering flippant excuses (*e.g.*, he slept through the meeting). *See* Correspondence Regarding Team Meeting, dated as of October 5, 2020 and November 6, 2020, attached hereto as **Exhibit Q**.

See also Decision to Terminate, executed as of November 19, 2020, attached hereto as **Exhibit R**. On November 19, 2020, in an internal document, Bradley McDonald (Director of Core Clinical Applications) summarized Mr. Proffitt's position with the Hospital:

> *Conor was provided with clear goals and expectations, which were reiterated to him in July. He continues to be unable to meet basic performance expectations. His inability to*

engage in completing important CHAMPS work that supports hospital operations has negatively affected team workflow, as well [as] created serious concerns regarding service delivery to clients. Conor lacks accountability and sustainability to meet overall performance expectations.

Id. Accordingly, Mr. Proffitt's employment with the Hospital was terminated, effective November 19, 2020. *Id.*

III. COMPLAINT'S ALLEGATIONS

The specific allegations contained in the Charge, as well as Respondent's response to each of these allegations, are set out below.

Allegation No. 1

I identify my race and color as White.

Response No. 1

With respect to Allegation No. 1, Respondent does not have sufficient information to respond. Respondent admits that Mr. Proffitt, as an employee of the Hospital, identified his race as White.

Allegation No. 2

In May 2019 started employment with Respondent as an Application analyst.

Response No. 2

Respondent admits that it hired Mr. Proffitt as an Electronic Medical Record Analyst on or about May 20, 2019. *See* **Exhibit C**.

Allegation No. 3

In January 2020, Travis Rossner (White), Manager, stated, "I don't see why we have to take Martin Luther King Day off" and "I wish we didn't have to take Martin Luther King Day off" in the presence of myself and Brad McDonald (White), Director of Core Clinical Applications and Rossner's supervisor.

Response No. 3

Respondent denies the allegations in Allegation No 3.

Allegation No. 4

In November 2019, I received a mid-year review from Rossner.

Response No. 4

Respondent admits that, on or about January 10, 2020, Mr. Rossner provided Mr. Proffitt with a mid-year review. *See* **Exhibit E**. In the meeting, Mr. Rossner confirmed that Mr. Proffitt's first six months of employment had presented serious concerns regarding interpersonal interactions, communication regarding key initiatives, and resolving projects that had been assigned to him.

Allegation No. 5

In June or July 2020, I received a yearly review from Rossner that I was not meeting performance goals.

Response No. 5

Respondent admits that on or about July 15, 2020, Mr. Rossner provided Mr. Proffitt with his performance review, which indicated that Mr. Proffitt "Need[ed] Immediate Improvement." **Exhibit I**. The key areas of improvement included his unprofessional and confrontational demeanor, poor communication, and an appropriate level of effort to resolve key projects. *Id.*

Allegation No. 6

In July 2020, I reported to Human Resources via phone and later felt I had to email Human Resources that Rossner made these comments related to Martin Luther King Day.

Response No. 6

Respondent admits that, on or about July 9, 2020, Mr. Proffitt called Human Resources and asserted that Mr. Rossner had made comments related to Martin Luther King Day. Further, Respondent admits that, on or about July 16, 2020, Mr. Proffitt filed a Grievance. *See* **Exhibit L**. Mr. Proffitt made this purported report approximately six months after Mr. Rossner's comments were allegedly made, and only after Mr. Proffitt had received his annual performance evaluation and a Final Written Warning. *See* **Exhibit K**.

Allegation No. 7

In July 2020, I joined an equity inclusion committee.

Response No. 7

With respect to Allegation No. 7, Respondent does not have sufficient information to respond.

Allegation No. 8

On November 19, 2020, I was terminated for performance reasons by Rossner.

Response No. 8

Respondent admits that, on or about November 19, 2020, Mr. Rossner indicated that the Hospital would have to terminate Mr. Proffitt's employment due to long-standing and well-documented performance concerns. *See* **Exhibit R**.

Allegation No. 9

I believe that following my complaint of discriminatory statements in the workplace based on race and color, I was terminated in retaliation. Therefore, I charge Respondent with discrimination on the basis of race and color and retaliation.

Response No. 9

Respondent denies the allegations in Allegation No. 9.

IV. ANALYSIS

Now, Mr. Proffitt alleges that the Hospital has discriminated against him on the basis of his race and retaliated against him. However, there is no evidence that BCH took *any* employment action due to any prohibited factor. Rather, the Hospital only terminated Mr. Proffitt's employment following a consistent, and well-documented, pattern of poor performance. Accordingly, the Charge should be dismissed for lack of probable cause.

A. Mr. Proffitt's Claim of Race and/or Color Discrimination Fails.

Mr. Proffitt's allegation of race-based discrimination is without factual or legal merit. "[T]he central focus of inquiry [in a discrimination case] is whether the employer penalizes" an employee "because of *their race*, color, religion, sex, or national origin." *Harvard Law Sch. Coal. for Civil Rights v. President & Fellows of Harvard Coll.*, 413 Mass. 66, 69 (1992) (emphasis added). To establish a valid employment discrimination claim under both Massachusetts and federal law, a complainant must assert that they were discriminated against on the basis of their personal racial identity, and not the identity of some third party. *See* 42 U.S.C. § 2000e–2 ("[i]t shall be an unlawful employment practice for an employer to . . . discharge any individual, . . . because of *such individual's race*"); M.G.L. c. 151B § 4(1) ("[i]t shall be an unlawful practice. . . for an employer. . . because of the race. . . of any individual to . . . discharge from employment such individual . . . ."). Accordingly, the central issue here is whether the Hospital discriminated against Mr. Proffitt based on *his* race or color as an individual who self-identifies as white. The Hospital did not.

Mr. Proffitt has offered no evidence or theory regarding how *his* race or color was actually involved in this case. The only relevant piece of information offered by the Complainant regarding racial discrimination in this case is when he asserts that he self-identifies as white. **Charge** ¶1. Yet, Mr. Proffitt has provided no absolutely no evidence that BCH discriminated against him on the basis of his identity as a white man. Moreover, Mr. Proffitt does not *allege* that anyone discriminated against him on the basis of his race. Thus, the Charge should be dismissed for lack of probable cause.

Mr. Proffitt has also not attempted to explain how *any* race or color contributed to any alleged discrimination in this case. At most, Mr. Proffitt asserts that his manager stated, "I don't see why we have to take Martin Luther King Day off" and "I wish we didn't have to take Martin Luther King Day off." **Charge** ¶3. Mr. Proffitt makes no attempt to describe how this alleged conversation ties in with his case of purported racial discrimination. Assuming *arguendo* that this conversation occurred as Mr. Proffitt describes it (which it did not), this appears to be a conversation about mandatory days off with the Hospital, and not about race. Courts have clearly stated that this type of "isolated, ambiguous, or stray remark is insufficient, standing alone, to prove discriminatory employment action." *Finney v. Madico, Inc.*, 42 Mass. App. Ct. 46, 51, 674 N.E.2d 655, 658 (1997). It seems that Mr. Proffitt is pointing to the alleged conversation about Martin Luther King Day in an attempt to raise the specter of racial discrimination against historically disadvantaged groups. If Mr. Proffitt is, in fact, trying to tie *his* racial discrimination allegation to the identifying characteristics of *another* group, this is legally impermissible. Perhaps more importantly, this strategy appears to be an offensively transparent attempt to use the racial identity of another group to create legal cover for himself after he was notified of his substandard performance. Mr. Proffitt's racial discrimination claim must fail.

Mr. Proffitt has also failed to satisfy the technical grounds of his *prima facie* burden. To establish a *prima facie* case of racial and/or color discrimination, Mr. Proffitt must show that "(1) he is a member of a protected class; (2) he was qualified for the job; (3) the [Hospital] took an adverse employment action against him; and (4) the position remained open or was filled by a person with similar qualifications." *Kosereis v. Rhode Island*, 331 F.3d 207, 212–13 (1st Cir. 2003). Here, Mr. Proffitt cannot satisfy his *prima face* burden and show that he was qualified for the job (prong two) or that the position was filled by a person with similar qualifications (prong four).

First, Mr. Proffitt has not demonstrated that he was able to perform the functions of his job (prong two). As described above, Mr. Proffitt was separated from his position after a continued pattern of significantly substandard performance issues. **Exhibit U**. Mr. Proffitt's employment was terminated only after Mr. Proffitt had already been counseled repeatedly for his failure to adequately complete projects, responsibly communicate about their progress, and professionally interact with peers and customers. *See infra* § II.B; II.D. Mr. Proffitt was diligently and appropriately moved through the Hospital's standard progressive discipline process, with Mr. Proffitt receiving verbal warnings, written warnings, and ultimately culminating in a PIN/Final Written Warning. *Id.* The Hospital simply could not allow Mr. Proffitt to continue to impact the quality and efficiency of the applications being used by our clinicians in the course of patient care. Ultimately, Mr. Proffitt was not qualified to perform the functions of his job, as demonstrated by his persistent performance concerns that were below the requirements of his position. *See* **Exhibits D-I**; **K**; **O-R**.

Critically, Mr. Proffitt has made absolutely no allegation regarding the final element of his prima facie case—Mr. Proffitt has not even alleged that the Hospital planned to replace him. Mr. Proffitt's failure to satisfy this burden is easily explainable: since Mr. Proffitt's separation from the Hospital, the Hospital has not hired any new Application Development Specialists IIIs. Mr. Proffitt has not satisfied his prima facie burden, and his discrimination claim should be dismissed because he has not identified *any* comparator or potential comparator regarding his position.

Ultimately, Mr. Proffitt does not offer sufficient evidence to establish that the Hospital took an adverse employment action taken against him because of his race. The uncontroverted evidence shows that the cause for Mr. Proffitt's termination was neutral, clear and wholly unrelated to race; the Hospital only terminated Mr. Proffitt's employment after engaging in the progressive discipline policy due to his persistent performance issues, which he refused to change despite numerous opportunities and structures put in place to help him. *See infra* § II.B. Mr. Proffitt's discrimination claims must be dismissed.

B. Mr. Proffitt's Retaliation Claim should be Dismissed.

Mr. Proffitt's retaliation claim should be dismissed for many of the same reasons that his discrimination claim should be dismissed. "To establish actionable retaliation, the plaintiff must show that [he] reasonably and in good faith believed that the employer had wrongfully discriminated and thereafter engaged in legally protected conduct, suffered an adverse employment action, and that a causal connection existed between the protected conduct and the

adverse action." *Cameron v. Dep't of State Police*, 60 Mass. App. Ct. 1122, (2004). Here, Mr. Proffitt cannot satisfy his prima facie case.

First, Mr. Proffitt failed to engage in any protected conduct. Under Massachusetts law, a plaintiff has engaged in protected activity if "he has opposed any practices forbidden by [M.G.L. c. 151B] or because he has filed a complaint, testified or assisted in any proceeding under [the statute]." M.G.L. c. 151B, §4. Here, even taking Mr. Proffitt's report to Human Resources at its word, he still did assert oppose any practices forbidden by M.G.L. c. 151B. At most, Proffitt asserted that Mr. Rossner stated that "I don't see why we have to take Martin Luther King Day off." **Charge** ¶ 3. Assuming Mr. Proffitt's version of events was accurate, which it was not, he has made an assertion that Mr. Rossner made a stray and ambiguous remark. This report does not suffice for the purposes of creating protected conduct because it does not assert that Mr. Rossner, or any individual at BCH, was engaged in a practice forbidden by M.G.L. c. 151B. Accordingly, the retaliation claim is without merit.

Mr. Proffitt's retaliation also fails because Mr. Proffitt has not established that he, *in good faith*, believed that Boston Children's Hospital was actually engaged in wrongful discrimination. The MCAD's Guidelines on Chapter 151B specifically provide that an employee who makes a claim not in good faith is not entitled to legal protection. *See* MCAD's "Guidelines on 151B", available at https://www.mass.gov/doc/mcad-guidelines-on-sexual-harassment-laws-in-employment/download (confirming that an employer may discipline an employee for making a bad faith report); see also EEOC-CVG-2016-1, *Enforcement Guidance on Retaliation and Related Issues*, (Aug. 25, 2016), available at https://www.eeoc.gov/laws/guidance/enforcement-guidance-retaliation-and-related-issues#_ftnref69 (recognizing that "[f]or statements or actions to be protected opposition, however, they must be based on a reasonable good faith belief that the conduct opposed violates the EEO laws, or could do so if repeated."). In fact, courts have been clear that an employee does not engage in good faith protected conduct when they file a complaint in response to receiving corrective action notice from their employer. *See, e.g.*, *Rae v. Massachusetts Bay Transportation Auth.*, No. CV 19-11044-PBS, 2021 WL 1580838, at *7 (D. Mass. Apr. 22, 2021) (finding that an employee could not "manufacture" a retaliation claim by filing a purported complaint after finding out he is under investigation).

Here, Mr. Proffitt filed his purported complaint to Human Resources *on the very day* that he received a Final Written Warning. The fact that Mr. Proffitt's purported complaint was about a conversation that allegedly occurred six months prior completely undercuts any assertion that Mr. Proffitt's genuinely believed that his supervisor, or anyone at the Hospital, was actually engaged in discriminatory conduct. Moreover, H.R. fully reviewed this complaint and found Mr. Proffitt's version of events to be fundamentally fabricated. In sum, Mr. Proffitt engaged in a transparent and baseless attempt to create legal cover for himself when he had just been notified that he was on his last chance with the Hospital. This is the exact type of *faux* "protected conduct" that courts have found to be unworthy of protection. The retaliation claim should be dismissed.

Mr. Proffitt has also failed to demonstrate *any type* of causal connection between his purported report to Human Resources and the termination of his employment. In fact, Mr. Proffitt has provided no evidence beyond a conclusory assertion that he "was terminated in retaliation." **Charge** ¶9. This conclusory assertion is insufficient as a matter of law. *See Pina v. Children's Place*, 740 F.3d 785, 796 (1st Cir. 2014) (finding dismissal appropriate in conclusory retaliation

pleading under Massachusetts law, as Complainant cannot rely "merely upon conclusory allegations, improbable inferences, and unsupported speculation.").

Moreover, "[o]n a claim of retaliatory discharge, unless the termination is very closely connected in time to the protected activity, the plaintiff must rely on additional evidence beyond temporal proximity to establish causation." *Mole v. Univ. of Massachusetts*, 442 Mass. 582, 595 (2004). Courts have clearly stated that a "gap of four months, on its own, is not 'very close' for establishing causality" in a retaliation case. *Pena v. Honeywell Int'l, Inc.*, 923 F.3d 18, 32 (1st Cir. 2019). Here, there was, in fact, a four-month period between Mr. Proffitt's report to HR; yet, the Complainant has not provided the required additional evidence. Mr. Proffitt's lack of "additional evidence" is easily explained: Mr. Proffitt was not in terminated for any reason connected to his fabricated H.R. complaint; he was terminated due to his failure to correct the critical performance issues that had been clearly communicated to him on a number of occasions. *See infra* §II.B; II.D.

Even if Mr. Proffitt's termination was found to be temporally "very closely connected in time" to his termination, his claim would still fail. Courts have explained that, when there is a close temporal nexus in a retaliation case, "intervening acts of misconduct sever the causal connection (if any) between the protected conduct and the adverse employment decision." *Furtado v. Standard Parking Corp.*, 820 F. Supp. 2d 261, 273 (D. Mass. 2011); *see also Whatley v. Metropolitan Atlanta Rapid Transit Auth.*, 632 F.2d 1325, 1329 (5th Cir.1980) (ruling that, even though the plaintiff filed a discrimination charge shortly before being terminated, the dismissal actually was caused by "a culmination of problems" with the plaintiff's job performance). Here, because Mr. Proffitt continued his pattern of poor performance after his HR report, *see infra.* §II.D, there clearly was no causal connection linking the HR report and his separation. Mr. Proffitt has not satisfied his *prima facie* burden and his retaliation claim must fail.

C. <u>The Hospital Has Articulated a Legitimate, Non-Discriminatory Reason for the Termination of Mr. Proffitt's Employment.</u>

Even assuming Mr. Proffitt has satisfied his *prima facie* burden for his race and/or color discrimination claim or his retaliation claim, which he has not, each of these claims still fails because the Hospital has articulated a legitimate, non-discriminatory reason for its employment decisions. *See Blare v. Husky Injection Molding Sys. Bos., Inc.*, 419 Mass. 437, 441 (1995) ("the employer can rebut the presumption created by the prima facie case by articulating a legitimate, nondiscriminatory reason for its . . . decision."). The Hospital has clearly articulated its reasons for Mr. Proffitt's termination: Mr. Proffitt struggled to adequately perform the responsibilities of his position due to his challenging interpersonal style; his disinterest in completing critical projects; and his lack of accountability about his failures. *See infra* § II.B; II.D. Mr. Proffitt's performance issues delayed the implementation of tools that assist our clinicians in the delivery of the best possible patient care. The Hospital terminated Mr. Proffitt's employment for these well-documented reasons, and, thus, Mr. Proffitt's claims fail.

D. <u>The Hospital's Commitment to Antiracism and Diversity & Inclusion.</u>

Mr. Proffitt's claims are particularly misplaced in the broader culture of Boston Children's Hospital. Boston Children's Hospital remains committed to the principles of health equity, diversity, inclusion and the promotion of anti-racist principles. *See* "About Boston Children's Hospital Website", attached hereto as **<u>Exhibit S</u>** (recognizing that the Hospital is "committed to working together to support health equity and promote anti-racist practices."). In fact, in August 2020, the Hospital announced "Boston Children's Declaration on Equity, Diversity, and

Inclusivity", which includes commitments to equity, diversity, and various anti-racist practices. *See* "Boston Children's Declaration on Equity, Diversity, and Inclusivity", attached hereto as **Exhibit T**. Mr. Proffitt's assertion that he was discriminated against, or retaliated against, by the leadership of his area, due to a purported report to Human Resources about MLK Day, is fundamentally unbelievable. Mr. Proffitt's claims are not just insufficient as a matter of fact and law, they are also unsupported as a matter of common sense. The Charge must be dismissed.

V. AFFIRMATIVE DEFENSES

Unless otherwise specified herein, Respondent denies, or lacks sufficient information to answer, each and every allegation in the Charge.

Respondent asserts the following defenses in response to the allegations contained in the Charge:

1. Complainant failed to establish a *prima facie* case with respect to his claims of discrimination and/or retaliation.
2. Complainant failed to file a clear and concise statement of alleged discriminatory acts that would allow the Respondent to sufficiently respond to the allegations.
3. Complainant failed to state a claim upon which relief may be granted.
4. Complainant's allegations concerning matters that occurred prior to the applicable statute of limitations are barred by the statute of limitations.
5. Complainant failed to establish a causal connection between his exercise of statutory rights and any purported adverse employment action.
6. Every action taken by Respondent with respect to Complainant's employment was taken for a legitimate business purpose and was consistent with the law.
7. The Hospital is a charitable corporation and therefore entitled to charitable immunity on some or all of these claims.

BCH reserves its rights to raise additional defenses which may become apparent during the course of the Commission's proceedings.

VI. CONCLUSION

Complainant has presented no evidence for his claims of discrimination or retaliation. Because Complainant cannot sustain any of his claims, Boston Children's Hospital respectfully requests that the Charge be dismissed for lack of probable clause.

I, upon information and belief, state under the pains and penalties of perjury, that I am authorized to verify this position statement on behalf of the Respondent, Boston Children's Hospital, and that while I do not have personal knowledge of all of the various facts recited in these answers, the response was prepared with the assistance and advice of counsel and information from the Respondent, upon which I have relied; and that the facts stated, subject to these limitations are true to the best of my knowledge, information and belief.

For the Respondent:

11/19/2021
Date

Scott Ogawa
VP for IT & Chief Technology Officer
Boston Children's Hospital
300 Longwood Avenue, Boston, MA 02115

Legal Counsel:

11/19/2021
Date

Benjamin O. Hoerner, Esq.
Assistant General Counsel
Office of the General Counsel
Boston Children's Hospital
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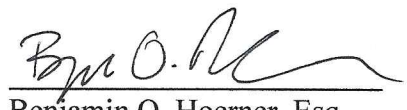
Boston, MA 02115
Phone: (617) 355-4934
Fax: (617) 730-1952

CERTIFICATE OF SERVICE

I, Benjamin O. Hoerner, Esq., attorney for Boston Children's Hospital, hereby certify that on November 19, 2021, I caused notice to be given of the within Position Statement to Complainant at the following address:

Conor L. Proffitt
4075 La Salle Avenue, Apt. B
Culver City, CA 90232



Benjamin O. Hoerner, Esq.